

Medical Report Analysis

Report Name:	Medical_Report_27
Analysis Date:	12 May 2026
Health Score:	21/100

Summary

2 critical, 4 out-of-range, 7 borderline parameters found. Health score: 21/100 (Critical).

Issues Detected

Parameter	Value	Status	Deviation
Vitamin D	12.5 ng/mL	Critical	58.3%
Bilirubin Indirect	0.91 mg/dL	Critical	51.7%
ESR	30.0 mm/hr	High	50.0%
Bilirubin Total	1.26 mg/dL	High	40.0%
HDL	38.4 mg/dL	Low	30.2%
Creatinine	0.45 mg/dL	Low	25.0%

Pattern Analysis

Iron Deficiency Anaemia

Haemoglobin, MCH and MCHC — all three are low. Classic pattern of Iron Deficiency Anaemia. RBCs become small due to lack of iron and carry less haemoglobin.

Action Required:

1. Start iron supplement (ask doctor). 2. Take with Vitamin C for better absorption. 3. Eat spinach, chickpeas, dates, meat. 4. Avoid tea/coffee with food. 5. Repeat CBC after 3 months.

Cardiovascular Risk Pattern

Triglycerides high, HDL (good cholesterol) low, VLDL high — this combination is a heart disease risk indicator.

Action Required:

1. Reduce sweets, refined flour and rice. 2. Walk 30-45 minutes daily. 3. Eat olive oil, nuts and avocado. 4. Consider Omega-3 — ask your doctor. 5. See a cardiologist.

Vitamin Deficiency

Vitamin D is deficient (deficiency zone <20). Very common in India, especially women above 40. Causes fatigue, bone pain, and low immunity.

Action Required:

1. Vitamin D3: 60,000 IU weekly for 8 weeks (confirm with doctor). 2. Then maintenance: 1000-2000 IU daily. 3. Morning sunlight 15-20 min without sunscreen. 4. For B12: eat yoghurt, milk, eggs, paneer. 5. Retest Vitamin D after 3 months.

Elevated Bilirubin — Liver Watch

Total and Indirect Bilirubin are slightly elevated. SGOT/SGPT are normal — so this is not liver cell damage. Could be Gilbert's Syndrome or mild haemolysis — usually benign but must be confirmed.

Action Required:

1. Stop alcohol completely. 2. Avoid paracetamol overuse. 3. Get a liver ultrasound. 4. Repeat LFT in 4-6 weeks. 5. Get Reticulocyte Count to rule out haemolysis.

Elevated Inflammation Marker (ESR)

ESR is high — normal for women is 0-20. ESR is non-specific — it shows inflammation somewhere in the body. Could be due to anaemia, infection, arthritis or autoimmune condition.

Action Required:

1. Treat anaemia first — iron deficiency can raise ESR. 2. Get a CRP test — more specific. 3. If joint pain, see a rheumatologist. 4. ESR must be seen in context.

Final Advice

Pay attention to 'Vitamin D' (Critical) first. Read the patterns and suggestions below carefully. Final diagnosis must be done by a doctor — this report is only a guide.

This report is AI-generated and for informational purposes only. Please consult a qualified medical professional for diagnosis and treatment.